



ICD-O-3
Carcinoma, Lobular NOS
8520/3
Site @ Breast NOS
250.9
JW 5/10/13

Surgical Pathology Report
* Amended *

DATE OBTAINED:
DATE RECEIVED:
DATE REPORTED:

LOCATION:
SUBMITTING MD:
CC:

MD

DIAGNOSIS

1. SUPERFICIAL RIGHT AXILLARY BREAST TISSUE, EXCISION: SMALL FOCUS OF INVASIVE LOBULAR CARCINOMA (0.6MM) PRESENT AT THE EDGE OF ONE FRAGMENT, WITH SURROUNDING ADIPOSE TISSUE; RARE BENIGN BREAST ACINI; SEE COMMENT

2. RIGHT AXILLARY SENTINEL LYMPH NODE #1: ONE LYMPH NODE POSITIVE FOR MICROMETASTASIS (0.6 MM); SEE COMMENT.

3. RIGHT AXILLARY SENTINEL LYMPH NODE #2: ONE LYMPH NODE POSITIVE FOR MULTIPLE FOCI OF MICROMETASTASIS (LARGEST 1.5MM), CONFIRMED BY CYTOKERATIN STAIN; NO EXTRANODAL EXTENSION.

4. RIGHT BREAST, MASTECTOMY: INVASIVE LOBULAR CARCINOMA

SIZE (INVASIVE): 3.5 cm
LATERALITY: Right
TUMOR FOCALITY: Unifocal
LESIONAL SITE: 10:00
HISTOLOGIC TYPE: Invasive lobular carcinoma, classical and pleomorphic types
NUCLEAR GRADE: II and III of III
HISTOLOGIC GRADE: N/A (invasive lobular carcinoma)
IN-SITU COMPONENT: LCIS (Classical and focal pleomorphic types; NG II, focal III)
LYMPH NODE SAMPLING: Positive (2/15), largest 3mm; no extranodal involvement; see specimens #2, #3 & #6
AJCC CATEGORIES: Stage IIB
pTNM: pT2 pN1mi
cTNM: cT2 cN1 cM0
INTEGRITY/ORIENTATION: Intact specimen with designated margins
MARGINS (invasive lobular): Focal positive superior/anterior margin (see final margin in #1);
Other margins negative (>3mm)
LYMPHOVASCULAR INVASION: Present
MICROCALCIFICATIONS: Not identified
NIPPLE/SKIN: Negative skin and nipple
SKELETAL MUSCLE: Present, focal, negative for tumor
OTHER: Focal sclerosing adenosis, columnar cell changes and usual mild hyperplasia

5. LEFT BREAST, PROPHYLACTIC MASTECTOMY: BENIGN BREAST WITH SMALL PAPILLOMATOSIS, SCLEROSING ADENOSIS AND MILD USUAL DUCTAL HYPERPLASIA; NO ATYPIA; NEGATIVE SKIN AND NIPPLE.

6. RIGHT AXILLARY CONTENTS, DISSECTION: THIRTEEN NEGATIVE LYMPH NODES (0/13).

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
ILPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Nodules		☒
Case is (circle): QUALIFIED / DISQUALIFIED		
Reviewer Initials: BJR Date Reviewed: 4/8/13		

Electronically Signed Out

COMMENT

#1: There is one small focus of invasive carcinoma at the edge of a fragment (0.6mm, confirmed by negative calponin and p63 immunostain), with severe cautery artefact, which was not seen on frozen section. The tumor is surrounded by fatty tissue, and thus would be regarded as negative final margin for specimen #4.

#2: There is single focus of micrometastasis in the SLN #1. This focus is only present in slide 2B CTD2, and not on other levels, frozen smears and cytokeratin immunostained slides.

88307x5, 88305, 88331, 88332, 88333x2, 88342x3

Clinical Diagnosis and History:

Right breast cancer 10 o'clock
cT2,cN1,cMo, clinical stage 2B

Tissue(s) Submitted:

- 1: SUPERFICIAL RIGHT AXILLARY BREAST TISSUE
- 2: RIGHT AXILLARY SENTINEL LYMPH NODE #1
- 3: RIGHT AXILLARY SENTINEL LYMPH NODE #2
- 4: RIGHT BREAST LONG STITCH LATERAL SHORT STITCH SUPERIOR
- 5: LEFT BREAST LONG STITCH LATERAL SHORT STITCH SUPERIOR
- 6: RIGHT AXILLARY CONTENTS

Gross Description:

Specimen #1 is received fresh for intraoperative consultation, labeled with the patient's name and superficial right axillary breast tissue, and consists of a 2.7 x 1.7 x 0.6 cm tan-yellow lobulated adipose tissue which is entirely submitted for frozen section analysis labeled 1AFS-1BFS.

Specimen #2 is received fresh for intraoperative consultation, labeled with the patient's name and right axillary sentinel lymph node #1, and consists of a single tan-pink, nodular tissue consistent with lymph node measuring 1.3 x 0.7 x 0.5 cm. The specimen is serially sectioned and two intraoperative smears are performed. The specimen is entirely submitted labeled 2A-2B.

Specimen #3 is received fresh on a blue surgical towel for intraoperative consultation, labeled with the patient's name and right axillary sentinel lymph node #2, and consists of a single tan-pink, nodular tissue consistent with lymph node measuring 1.0 x 0.5 x 0.5 cm. The specimen is serially sectioned and an intraoperative smear is prepared. The specimen is entirely submitted labeled 3A-3B.

Specimen #4 is received fresh for formalin, labeled with the patient's name and right breast, long stitch lateral, short stitch superior, and consists of a 520 gram, 18 (superior to inferior) by 17 (medial to lateral) by 4 (anterior to posterior) cm right mastectomy specimen. A short suture designating superior, and a long suture designating lateral are present. A 14 x 4.5 x 0.3 cm white-tan, unremarkable ellipse of skin is identified containing a 1.3 x 1.1 x 0.3 cm unremarkable, everted nipple. The areola measures 3.5 x 2.5 cm. The specimen is differentially inked as follows: superior/anterior – blue, inferior/anterior – green, posterior/deep – black and serially sectioned from lateral to medial to reveal a 3.5 x 2.5 x 1.1 cm firm, tan-pink, irregular mass at the 10 o'clock position. This mass is focally 0.2 cm from the posterior, 0.2 cm from the superior/anterior, and greater than 4 cm from the inferior margins. The mass is present in multiple sections and is seen to extend more centrally. No other lesions are grossly identified. The surrounding breast parenchyma consists of white, fibrous tissue (60%) and tan lobular adipose tissue (40%). Time in formalin: 14:20. Representative sections are submitted as follows:

- 4A-4B: Mass closest to superior/anterior and deep margins, contiguous section
- 4C: Mass to deep margin
- 4D: Mass to superior/anterior margin
- 4E: Mass to deep margin
- 4F-4G: Tumor without margin
- 4H: Mass to deep
- 4I: Tumor without margin
- 4J: Inferior margin closest to tumor
- 4K-4L: Fibrous tissue immediately medial to mass
- 4M: Fibrous tissue in central breast
- 4N: Fibrous tissue in medial most breast

4O: Nipple and skin section closest to mass
4P: Outer upper quadrant
4Q: Outer lower quadrant
4R: Inner upper quadrant
4S: Inner lower quadrant

Specimen #5 is received fresh for formalin, labeled with the patient's name and left breast, long stitch lateral, short stitch superior, and consists of a 550-gram, 20 (superior to inferior) by 15.9 (medial to lateral) by 5.4 (anterior to posterior) product of left mastectomy. An unremarkable, tan ellipse of skin is identified measuring 13.3 x 4.2 x 0.2 cm. The everted nipple measures 1 x 0.9 cm and resides within a 4 x 2.9 cm areola. The specimen is differentially inked as follows: superior/anterior – blue, inferior/anterior – green, posterior/deep – black and serially sectioned from medial to lateral to reveal no distinct mass but areas consistent with fibrocystic changes containing blue domed cysts averaging 0.3 cm in greatest dimension. The surrounding breast parenchyma consists of tan-yellow lobulated adipose tissue (60%) and white, fibrous areas (40%). No other lesions are grossly identified. Time in formalin: Representative sections are submitted as follows:

5A: Nipple
5B: Outer upper quadrant
5C: Inner upper quadrant
5D: Inner lower quadrant
5E: Outer lower quadrant
5F-5L: Fibrous areas
5M: Superior margin to fibrous area
5N: Deep margin to fibrous area
5O: Inferior margin to fibrous area

Specimen #6 is received fresh on a blue surgical towel, labeled with the patient's name and right axillary contents, and consists of multiple tan-pink, nodular tissues consistent with lymph nodes ranging from 0.5 to 2 cm in greatest dimension. All grossly identifiable lymph nodes are submitted as follows:

6A-B: One lymph node, bisected, each cassette
6C-6D: One lymph node, bisected
6E-F: One lymph node, bisected, each cassette
6G-H: Intact lymph nodes

Intraoperative Consult Diagnosis

1A-1B/FSDX: BENIGN BREAST TISSUE AND FAT; NO MALIGNANCY.
2A/GDX/SMDX: NEGATIVE LYMPH NODE (0/1)
3A/GDX/SMDX: POSITIVE LYMPH NODE (1/1).

Amendments

Amended:

Reason: Further review of case

SLN #2 should be counted as multiple micromet

Previous Signout Date: